



ARIZONA HOUSE OF REPRESENTATIVES FLOOR AMENDMENT EXPLANATION

57th Legislature, 2nd Regular Session

Majority Research Staff

SB 1713: AHCCCS; procurement; contracting

LOPEZ FLOOR AMENDMENT

1. Requires each pharmacy where testing and treatment are being provided to ensure that there is an area to provide privacy for patients.
2. Requires all Advisory Committee members to be appointed within 60 days after the general effective date.
3. Clarifies that if some Advisory Committee members are not appointed within 60 days, the members who have been appointed can still meet and make recommendations to the Pharmacy Board on the statewide written protocols.
4. Requires the Pharmacy Board to provide a copy of the statewide written protocols to the chairpersons of the Health & Human Services Committee of the House of Representatives and Arizona State Senate, or their successor committees and post the protocols on its public website.
5. Makes technical changes.

LOPEZ FLOOR AMENDMENT
HOUSE OF REPRESENTATIVES AMENDMENTS TO S.B. 1713
(Reference to APPROPRIATIONS S/E Committee amendment)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

~~[GREEN STRIKEOUT IN BRACKETS]~~ indicates new text removed from statute or previously enacted session law.

~~[Green strikeout in brackets]~~ indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<~~Green strikeout in carets~~>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Strike everything after the enacting clause and insert:

3 "Section 1. Title 32, chapter 18, article 3, Arizona Revised
4 Statutes, is amended by adding sections 32-1979.04, 32-1979.05 and
5 32-1979.06, to read:

6 32-1979.04. Pharmacists; statewide written protocol;
7 independent testing; treatment; health
8 conditions; notification and insurance
9 requirements

10 A. PURSUANT TO A STATEWIDE WRITTEN PROTOCOL APPROVED BY THE BOARD,
11 A PHARMACIST MAY INDEPENDENTLY ORDER, PERFORM AND INTERPRET TESTS THAT ARE
12 AUTHORIZED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION AND WAIVED
13 UNDER THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988 (P.L.
14 100-578; 102 STAT. 2903; 42 UNITED STATES CODE SECTION 201). A PHARMACIST
15 MAY INDEPENDENTLY INITIATE TREATMENT TO ELIGIBLE PERSONS WHO ARE AT LEAST
16 TWELVE YEARS OF AGE OR THE AGE AUTHORIZED BY THE TREATMENT, WHICHEVER AGE
17 IS OLDER, AND WHO HAVE TEST RESULTS THAT INDICATE THE NEED FOR TREATMENT,
18 BY A TEST THAT IS AUTHORIZED BY THE UNITED STATES FOOD AND DRUG
19 ADMINISTRATION AND WAIVED UNDER THE CLINICAL LABORATORY IMPROVEMENT
20 AMENDMENTS OF 1988, FOR ANY OF THE FOLLOWING:

- 21 1. INFLUENZA.
- 22 2. GROUP A STREPTOCOCCUS PHARYNGITIS.
- 23 3. SARS-COV-2 OR ANY OTHER CORONAVIRUS RESPIRATORY ILLNESS.
- 24 4. A CONDITION RELATED TO AN EMERGING OR EXISTING PUBLIC HEALTH
25 THREAT IDENTIFIED BY THE DEPARTMENT OF HEALTH SERVICES FOR WHICH A
26 STATEWIDE STANDING ORDER, RULE OR EXECUTIVE ORDER IS ISSUED.

1 B. WHEN DEVELOPING THE STATEWIDE WRITTEN PROTOCOL, THE BOARD SHALL
2 ADDRESS AT A MINIMUM THE FOLLOWING:
3 1. DOCUMENTATION.
4 2. RECORDS RETENTION.
5 3. REFERRALS.
6 4. PATIENT SCREENING REQUIREMENTS AND OBTAINING RELEVANT MEDICAL
7 HISTORY.
8 5. EXCLUSION CRITERIA.
9 6. TREATMENT INSTRUCTIONS BASED ON THE PATIENT'S AGE AND MEDICAL
10 HISTORY.
11 7. FOLLOW-UP MAINTENANCE AND CARE PLANS.
12 8. PHARMACIST TRAINING AND CERTIFICATION REQUIREMENTS.
13 C. A PHARMACIST WHO ORDERS OR CONDUCTS TESTING OR TREATS HEALTH
14 CONDITIONS PURSUANT TO SUBSECTION A OF THIS SECTION SHALL USE ANY TEST
15 THAT MAY GUIDE CLINICAL DECISION-MAKING FOR WHICH A WAIVER HAS BEEN
16 OBTAINED UNDER THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988, OR
17 THE FEDERAL RULES ADOPTED THEREUNDER, OR ANY SCREENING PROCEDURE THAT IS
18 ESTABLISHED BY THE STATEWIDE WRITTEN PROTOCOL.
19 D. A PHARMACIST SHALL USE EVIDENCE-BASED CLINICAL GUIDELINES
20 PUBLISHED BY THE UNITED STATES CENTERS FOR DISEASE CONTROL AND PREVENTION
21 OR THE INFECTIOUS DISEASES SOCIETY OF AMERICA, THE AMERICAN ACADEMY OF
22 PEDIATRICS COMMITTEE ON INFECTIOUS DISEASE OR ANOTHER CLINICALLY
23 RECOGNIZED RECOMMENDATION THAT ALIGNS WITH STANDARDS OF CARE IN PROVIDING
24 PATIENT TREATMENT PURSUANT TO SUBSECTION A OF THIS SECTION.
25 E. AN ELIGIBLE PERSON MUST MEET CRITERIA FOR TREATMENT BASED ON THE
26 STATEWIDE WRITTEN PROTOCOL THAT SPECIFIES THE FOLLOWING:
27 1. PATIENT INCLUSION AND EXCLUSION CRITERIA.
28 2. EXPLICIT MEDICAL REFERRAL CRITERIA.
29 F. A PHARMACIST SHALL REFER A PATIENT TO THE PATIENT'S PRIMARY CARE
30 PROVIDER, IF ONE IS IDENTIFIED, OR RECOMMEND FOLLOW UP WITH A PRIMARY CARE
31 PROVIDER, IF THE PATIENT EITHER:
32 1. IS NOT ELIGIBLE FOR TREATMENT PURSUANT TO THIS SECTION AND
33 PRESENTS WITH SYMPTOMS.
34 2. DOES NOT RESPOND TO THE INITIAL TREATMENT PROVIDED PURSUANT TO
35 THIS SECTION.
36 G. A PHARMACIST WHO INITIATES A TREATMENT UNDER THIS SECTION SHALL:
37 1. NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER, IF ONE IS
38 IDENTIFIED, WITHIN SEVENTY-TWO HOURS AFTER INITIATING TREATMENT PURSUANT
39 TO THIS SECTION. THE NOTICE SHALL INCLUDE THE PATIENT'S NAME, THE
40 TREATMENT INITIATED AND THE DATE OF TREATMENT AND MAY BE SUBMITTED BY
41 ENTRY INTO AN ELECTRONIC HEALTH RECORD OR BY TELEPHONE, FAX, MAIL OR
42 EMAIL. THE PHARMACIST SHALL MAKE A REASONABLE EFFORT TO IDENTIFY THE
43 PATIENT'S PRIMARY CARE PROVIDER BY AT LEAST ONE OF THE FOLLOWING METHODS:
44 (a) CHECKING PHARMACY RECORDS.
45 (b) REQUESTING THE INFORMATION FROM THE PATIENT OR, FOR A PATIENT
46 UNDER EIGHTEEN YEARS OF AGE, THE PATIENT'S PARENT OR GUARDIAN.
47 2. MAINTAIN FOR AT LEAST SEVEN YEARS A RECORD OF THE RESULTS OF ANY
48 TESTING OR SCREENING FOR WHICH A TREATMENT IS INITIATED PURSUANT TO THIS

1 SECTION, INCLUDING A SUMMARY OF THE VISIT, PATIENT ASSESSMENT INFORMATION,
2 HISTORY OF ILLNESS, EXAMINATION FINDINGS, VITALS AND PLAN OF CARE.

3 3. NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER, IF ONE IS
4 IDENTIFIED, WITHIN FORTY-EIGHT HOURS AFTER THE OCCURRENCE OF ANY ADVERSE
5 REACTION THAT IS REPORTED TO OR WITNESSED BY THE PHARMACIST AS A RESULT OF
6 THE TREATMENT PROVIDED PURSUANT TO THIS SECTION.

7 4. PROVIDE INFORMATIONAL MATERIALS TO THE PATIENT REQUESTING
8 TREATMENT OR, FOR A PATIENT UNDER EIGHTEEN YEARS OF AGE, TO THE PATIENT'S
9 PARENT OR GUARDIAN ABOUT THE IMPORTANCE OF PEDIATRIC PREVENTIVE HEALTH
10 CARE VISITS AS RECOMMENDED BY THE AMERICAN ACADEMY OF PEDIATRICS.

11 H. A PHARMACIST MAY DELEGATE THE TASK OF PERFORMING A TEST WAIVED
12 BY THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988 TO A LICENSED
13 MEMBER OF THE PHARMACY STAFF WHO IS UNDER THE SUPERVISION OF THE
14 PHARMACIST. A PHARMACIST MAY NOT DELEGATE ANY TASKS THAT INCLUDE CLINICAL
15 JUDGMENT OR TREATMENT AND MAY DELEGATE ONLY ANCILLARY DUTIES AS ALLOWED BY
16 BOARD RULES.

17 I. THIS SECTION DOES NOT REQUIRE A PHARMACIST TO PROVIDE THE
18 SERVICES AUTHORIZED BY THIS SECTION. PURSUANT TO SECTION 32-1901.01,
19 SUBSECTION A, PARAGRAPH 21, AN EMPLOYER MAY NOT OVERRULE A PHARMACIST'S
20 DECISION TO NOT PROVIDE SERVICES THAT ARE AUTHORIZED BY THIS SECTION.

21 J. THIS SECTION DOES NOT ESTABLISH A CAUSE OF ACTION AGAINST A
22 PATIENT'S PRIMARY CARE PROVIDER FOR ANY ADVERSE REACTION, COMPLICATION OR
23 NEGATIVE OUTCOME ARISING FROM ANY TREATMENT INITIATED BY A PHARMACIST
24 PURSUANT TO THIS SECTION.

25 K. A PHARMACIST MAY NOT INDEPENDENTLY INITIATE A TREATMENT USING
26 OPIOIDS FOR A PATIENT.

27 L. A PHARMACIST MAY NOT INDEPENDENTLY ORDER A TEST OR SCREENING OR
28 TREAT A MINOR WITHOUT THE WRITTEN CONSENT OF THE MINOR'S PARENT OR
29 GUARDIAN.

30 M. A PHARMACY SHALL DISPLAY A NOTICE AND INCLUDE IN A PATIENT'S
31 CONSENT PAPERWORK THAT THE TESTING AND TREATMENT BEING PERFORMED PURSUANT
32 TO THIS SECTION ARE BEING PERFORMED BY A PHARMACIST WITHOUT CONSULTATION
33 WITH OR OVERSIGHT BY A PHYSICIAN AND THAT THE PATIENT SHOULD CONSULT WITH
34 A PRIMARY CARE PROVIDER IF SYMPTOMS CONTINUE.

35 [N. EACH PHARMACY WHERE TESTING AND TREATMENT SERVICES ARE PROVIDED
36 PURSUANT TO THIS SECTION SHALL ENSURE THAT THERE IS AN AREA THAT PROVIDES
37 PRIVACY TO PATIENTS FOR THE TESTING AND TREATMENT SERVICES.]

38 [N.] [Q.] A PHARMACIST SHALL MAINTAIN PROFESSIONAL LIABILITY
39 INSURANCE IF PERFORMING SERVICES THAT ARE AUTHORIZED UNDER THIS SECTION.

40 32-1979.05. Independent testing and treatment advisory
41 committee; duties; members

42 A. THE BOARD SHALL ESTABLISH THE INDEPENDENT TESTING AND TREATMENT
43 ADVISORY COMMITTEE TO ASSIST THE BOARD IN DEVELOPING THIS STATE'S WRITTEN
44 PROTOCOLS RELATING TO PHARMACISTS' INDEPENDENT AUTHORITY TO ORDER TESTING
45 AND INITIATE TREATMENTS PURSUANT TO SECTIONS 32-1979.04 AND 32-1979.06.
46 THE ADVISORY COMMITTEE SHALL ALSO MAKE RECOMMENDATIONS TO THE BOARD
47 REGARDING THE [STATEWIDE WRITTEN] PROTOCOLS REQUIRED PURSUANT TO THAT
48 SECTION.

49 B. THE ADVISORY COMMITTEE SHALL INCLUDE AT LEAST THE FOLLOWING:

1 1. TWO PHARMACISTS WHO ARE LICENSED PURSUANT TO THIS CHAPTER AND
2 WHO ARE APPOINTED BY THE BOARD.

3 2. TWO PHYSICIANS WHO ARE LICENSED PURSUANT TO THIS TITLE, ONE WHO
4 SPECIALIZES IN PRIMARY CARE AND ONE WHO PRACTICES IN AN EMERGENCY MEDICINE
5 SETTING, AND WHO ARE APPOINTED BY THE ARIZONA MEDICAL BOARD OR THE ARIZONA
6 BOARD OF OSTEOPATHIC EXAMINERS IN MEDICINE AND SURGERY. AT LEAST ONE OF
7 THESE MEMBERS MUST HAVE A PATIENT POPULATION THAT IS SUBSTANTIALLY
8 COMPOSED OF CHILDREN AND ADOLESCENTS.

9 3. ONE PERSON WHO REPRESENTS A NONPROFIT PATIENT ADVOCACY
10 ORGANIZATION AND WHO IS APPOINTED BY THE GOVERNOR.

11 4. ONE NURSE PRACTITIONER WHO IS LICENSED PURSUANT TO CHAPTER 15 OF
12 THIS TITLE, WHO SPECIALIZES IN PRIMARY CARE OR EMERGENCY MEDICINE, WHO IS
13 ABLE TO PRESCRIBE MEDICATION AND WHO IS APPOINTED BY THE ARIZONA STATE
14 BOARD OF NURSING.

15 [C. ALL ADVISORY COMMITTEE MEMBERS SHALL BE APPOINTED WITHIN SIXTY
16 DAYS AFTER THE EFFECTIVE DATE OF THIS SECTION. IF A MEMBER OR MEMBERS ARE
17 NOT APPOINTED TO THE ADVISORY COMMITTEE WITHIN THAT TIME FRAME, THE OTHER
18 MEMBERS OF THE ADVISORY COMMITTEE MAY MEET AND MAKE RECOMMENDATIONS TO THE
19 BOARD RELATING TO THE STATEWIDE WRITTEN PROTOCOLS WITHOUT THE ADDITIONAL
20 MEMBER OR MEMBERS.]

21 ~~[C.]~~ [D.] ADVISORY COMMITTEE MEMBERS ARE NOT ELIGIBLE FOR
22 COMPENSATION OR REIMBURSEMENT OF EXPENSES.

23 ~~[D.]~~ [E.] THE ADVISORY COMMITTEE SHALL REVIEW THE STATEWIDE
24 [WRITTEN] PROTOCOLS ANNUALLY AND UPDATE WHEN NECESSARY.

25 [F. THE BOARD SHALL PROVIDE A COPY OF THE STATEWIDE WRITTEN
26 PROTOCOLS TO THE CHAIRPERSONS OF THE HEALTH AND HUMAN SERVICES COMMITTEES
27 OF THE HOUSE OF REPRESENTATIVES AND THE SENATE, OR THEIR SUCCESSOR
28 COMMITTEES, AND SHALL POST THE STATEWIDE WRITTEN PROTOCOLS ON THE BOARD'S
29 PUBLIC WEBSITE.]

30 32-1979.06. Human immunodeficiency virus prevention;
31 preexposure prophylaxis; postexposure
32 prophylaxis; definitions

33 A. IN ADDITION TO THE AUTHORITY PROVIDED IN SECTION 32-1979.04, A
34 PHARMACIST WHO IS LICENSED PURSUANT TO THIS CHAPTER MAY INITIATE AND
35 DISPENSE HUMAN IMMUNODEFICIENCY VIRUS PREEXPOSURE PROPHYLAXIS AND
36 POSTEXPOSURE PROPHYLAXIS PURSUANT TO THE STATEWIDE WRITTEN PROTOCOL
37 APPROVED BY THE BOARD.

38 B. BEFORE INITIATING A PREEXPOSURE PROPHYLAXIS, A PHARMACIST SHALL:
39 1. CONDUCT OR OBTAIN A CLINICAL LABORATORY IMPROVEMENT
40 AMENDMENTS-WAIVED HUMAN IMMUNODEFICIENCY VIRUS TEST AND CONFIRM A NEGATIVE
41 TEST RESULT.

42 2. ASSESS THE PATIENT FOR CONTRAINDICATIONS, DRUG INTERACTIONS AND
43 CLINICAL ELIGIBILITY CONSISTENT WITH THE CURRENT GUIDELINES OF THE UNITED
44 STATES CENTERS FOR DISEASE CONTROL AND PREVENTION.

45 C. BEFORE INITIATING A POSTEXPOSURE PROPHYLAXIS, A PHARMACIST
46 SHALL:

47 1. DETERMINE THAT THE PATIENT PRESENTS WITHIN SEVENTY-TWO HOURS
48 AFTER A POTENTIAL EXPOSURE TO THE HUMAN IMMUNODEFICIENCY VIRUS.

1 2. CONDUCT OR OBTAIN A HUMAN IMMUNODEFICIENCY VIRUS TEST IF
2 FEASIBLE UNDER THE STATEWIDE WRITTEN PROTOCOL.
3 3. ASSESS THE PATIENT FOR CONTRAINDICATIONS, DRUG INTERACTIONS AND
4 CLINICAL ELIGIBILITY CONSISTENT WITH THE CURRENT GUIDELINES OF THE UNITED
5 STATES CENTERS FOR DISEASE CONTROL AND PREVENTION.
6 D. A PHARMACIST WHO INITIATES THERAPY PURSUANT TO THIS SECTION:
7 1. MAY DISPENSE NOT MORE THAN A THIRTY-DAY SUPPLY OF PREEXPOSURE
8 PROPHYLAXIS.
9 2. MAY DISPENSE A COMPLETE TWENTY-EIGHT-DAY COURSE OF POSTEXPOSURE
10 PROPHYLAXIS.
11 3. SHALL PROVIDE COUNSELING REGARDING MEDICATION ADHERENCE,
12 POTENTIAL SIDE EFFECTS AND PREVENTION MEASURES.
13 4. SHALL PROVIDE THE PATIENT WITH A WRITTEN REFERRAL TO A PRIMARY
14 CARE PROVIDER OR OTHER APPROPRIATE HEALTH CARE PROVIDER FOR FOLLOW-UP
15 CARE.
16 5. SHALL NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER OF THE SERVICES
17 PROVIDED IF THE PATIENT CONSENTS TO THE NOTIFICATION.
18 E. THE STATEWIDE WRITTEN PROTOCOL APPROVED BY THE BOARD SHALL
19 ADDRESS, AT A MINIMUM, THE FOLLOWING:
20 1. MINIMUM TRAINING REQUIREMENTS FOR PHARMACISTS WHO INITIATE
21 THERAPY PURSUANT TO THIS SECTION.
22 2. DOCUMENTATION AND RECORDS RETENTION REQUIREMENTS.
23 3. STATEWIDE WRITTEN PROTOCOL STANDARDS THAT ARE CONSISTENT WITH
24 THE CURRENT GUIDELINES OF THE UNITED STATES CENTERS FOR DISEASE CONTROL
25 AND PREVENTION OR THE INFECTIOUS DISEASES SOCIETY OF AMERICA OR ANOTHER
26 CLINICALLY RECOGNIZED RECOMMENDATION.
27 F. FOR THE PURPOSES OF THIS SECTION:
28 1. "POSTEXPOSURE PROPHYLAXIS" MEANS A MEDICATION REGIMEN THAT IS
29 APPROVED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION AND THAT IS
30 INITIATED AFTER POTENTIAL EXPOSURE TO THE HUMAN IMMUNODEFICIENCY VIRUS TO
31 PREVENT INFECTION.
32 2. "PREEXPOSURE PROPHYLAXIS" MEANS A MEDICATION REGIMEN THAT IS
33 APPROVED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION TO PREVENT
34 HUMAN IMMUNODEFICIENCY VIRUS INFECTION IN PERSONS WHO ARE AT RISK."

35 Enroll and engross to conform

36 Amend title to conform

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C: MH

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